AAPC: CPT Professional 2026
aapc.vitalsource.com/reader/books/A26BPL0005/epubcfi/6/158[%3Bvnd.vst.idref%3Dt58_Appendix_A]!/4/2[Appendix_A]/2/4
Back
Search across book

Appendix A

Modifiers

This list includes all of the modifiers applicable to CPT 2026 codes.

A modifier provides the means to report or indicate that a service or procedure that has been performed has been altered by some specific circumstance but not changed in its definition or code. Modifiers also enable health care professionals to effectively respond to payment policy requirements established by other entities.

22	**Increased Procedural Services:** When the work required to provide a service is substantially greater than typically required, it may be identified by adding modifier 22 to the usual procedure code. Documentation must support the substantial additional work and the reason for the additional work (ie, increased intensity, time, technical difficulty of procedure, severity of patient's condition, physical and mental effort required). **Note:** This modifier should not be appended to an E/M service. *CPT Changes: An Insider's View* 2008 *CPT Assistant* Jan 09:8, Apr 09:8, Jun 09:8, 10, Aug 13:4, Dec 22:20, Jan 23:32, Feb 23:13, Mar 23:34, Nov 23:25, Dec 23:27, Jan 24:17, Feb 24:33, Apr 24:35, May 24:23 *Clinical Examples in Radiology* Spring 21:12
23	**Unusual Anesthesia:** Occasionally, a procedure, which usually requires either no anesthesia or local anesthesia, because of unusual circumstances must be done under general anesthesia. This circumstance may be reported by adding modifier 23 to the procedure code of the basic service.
24	**Unrelated Evaluation and Management Service by the Same Physician or Other Qualified Health Care Professional During a Postoperative Period:** The physician or other qualified health care professional may need to indicate that an evaluation and management service was performed during a postoperative period for a reason(s) unrelated to the original procedure. This circumstance may be reported by adding modifier 24 to the appropriate level of E/M service. *CPT Changes: An Insider's View* 2013
25	**Significant, Separately Identifiable Evaluation and Management Service by the Same Physician or Other Qualified Health Care Professional on the Same Day of the Procedure or Other Service:** It may be necessary to indicate that on the day a procedure or service identified by a CPT code was performed, the patient's condition required a significant, separately identifiable E/M service above and beyond the other service provided or beyond the usual preoperative and postoperative care associated with the procedure that was performed. A significant, separately identifiable E/M service is defined or substantiated by documentation that satisfies the relevant criteria for the respective E/M service to be reported (see **Evaluation and Management Services Guidelines** for instructions on determining level of E/M service). The E/M service may be prompted by the symptom or condition for which the procedure and/or service was provided. As such, different diagnoses are not required for reporting of the E/M services on the same date. This circumstance may be reported by adding modifier 25 to the appropriate level of E/M service. **Note:** This modifier is not used to report an E/M service that resulted in a decision to perform surgery. See modifier 57. For significant, separately identifiable non-E/M services, see modifier 59. *CPT Changes: An Insider's View* 2008, 2013 *CPT Assistant* Feb 09:22, Mar 09:3, Apr 09:4, Jun 09:11, Mar 23:1, Jul 23:1, Jan 24:1, Sep 24:8, Apr 25:25 *Clinical Examples in Radiology* Spring 23:26
26	**Professional Component:** Certain procedures are a combination of a physician or other qualified health care professional component and a technical component. When the physician or other qualified health care professional component is reported separately, the service may be identified by adding modifier 26 to the usual procedure number. *CPT Changes: An Insider's View* 2013 *CPT Assistant* Jan 09:7, Apr 09:4, May 09:7 *Clinical Examples in Radiology* Summer 22:10, Fall 23:5, Fall 24:19
32	**Mandated Services:** Services related to *mandated* consultation and/or related services (eg, third party payer, governmental, legislative or regulatory requirement) may be identified by adding modifier 32 to the basic procedure. *CPT Assistant* Aug 13:12
33	►**Preventive Services:** When the primary purpose of the service is the delivery of an evidence-based service in accordance with a US Preventive Services Task Force A or B rating in effect or is another preventive service identified in preventive services mandates (legislative or regulatory), the service may be identified by adding modifier 33 to the procedure. For separately reported services specifically identified as preventive, the modifier should not be used.◄ *CPT Changes: An Insider's View* 2012, 2026 *CPT Assistant* Sep 24:8
47	**Anesthesia by Surgeon:** Regional or general anesthesia provided by the surgeon may be reported by adding modifier 47 to the basic service. (This does not include local anesthesia.) **Note:** Modifier 47 would not be used as a modifier for the anesthesia procedures.
50	**Bilateral Procedure:** Unless otherwise identified in the listings, bilateral procedures that are performed at the same session should be identified by adding modifier 50 to the appropriate 5 digit code. **Note:** This modifier should not be appended to designated "add-on" codes (see Appendix D). *CPT Changes: An Insider's View* 2011, 2020 *CPT Assistant* Apr 09:9, Jun 13:15, Dec 22:20, Mar 23:32, May 23:28, Jul 23:18, Aug 23:14, Sep 23:1, Oct 23:19, Jan 24:17, Feb 24:34, May 24:7, Aug 24:1, Dec 24:58, Feb 25:29 *Clinical Examples in Radiology* Winter 23:8, Fall 23:32, Winter 24:17, Spring 24:3,25, Summer 24:3, Fall 24:4, 12, Winter 25:19
51	**Multiple Procedures:** When multiple procedures, other than E/M services, Physical Medicine and Rehabilitation services or provision of supplies (eg, vaccines), are performed at the same session by the same individual, the primary procedure or service may be reported as listed. The additional procedure(s) or service(s) may be identified by appending modifier 51 to the additional procedure or service code(s). **Note:** This modifier should not be appended to designated "add-on" codes (see Appendix D). *CPT Changes: An Insider's View* 2008, 2013 *CPT Assistant* Feb 09:6, Mar 09:10, Apr 09:8, Apr 23:26, Jul 23:1, Oct 23:19, Dec 23:39, Jan 24:17, Dec 24:56, Feb 25:21, Mar 25:19 *Clinical Examples in Radiology* Spring 21:3

52	**Reduced Services:** Under certain circumstances a service or procedure is partially reduced or eliminated at the discretion of the physician or other qualified health care professional. Under these circumstances the service provided can be identified by its usual procedure number and the addition of modifier 52, signifying that the service is reduced. This provides a means of reporting reduced services without disturbing the identification of the basic service. **Note:** For hospital outpatient reporting of a previously scheduled procedure/service that is partially reduced or cancelled as a result of extenuating circumstances or those that threaten the well-being of the patient prior to or after administration of anesthesia, see modifiers 73 and 74 (see modifiers approved for ASC hospital outpatient use). *CPT Changes: An Insider's View* 2013 *CPT Assistant* Mar 09:11, Apr 09:5, May 09:8, Jun 09:10, Jun 13:17, Nov 22:22, Dec 22:20, Feb 23:6, May 23:28, Jun 23:25, Dec 23:23, Jan 24:17, Apr 24:24, Jul 24:15, Aug 24:14, Sep 24:26, Oct 24:14, Dec 24:53, Jan 25:32 *Clinical Examples in Radiology* Summer 23:23, Winter 24:30, Summer 24:7, Winter 25:27
53	**Discontinued Procedure:** Under certain circumstances, the physician or other qualified health care professional may elect to terminate a surgical or diagnostic procedure. Due to extenuating circumstances or those that threaten the well being of the patient, it may be necessary to indicate that a surgical or diagnostic procedure was started but discontinued. This circumstance may be reported by adding modifier 53 to the code reported by the individual for the discontinued procedure. **Note:** This modifier is not used to report the elective cancellation of a procedure prior to the patient's anesthesia induction and/or surgical preparation in the operating suite. For outpatient hospital/ambulatory surgery center (ASC) reporting of a previously scheduled procedure/service that is partially reduced or cancelled as a result of extenuating circumstances or those that threaten the well being of the patient prior to or after administration of anesthesia, see modifiers 73 and 74 (see modifiers approved for ASC hospital outpatient use). *CPT Changes: An Insider's View* 2013 *CPT Assistant* Feb 14:11, Jan 23:31, Dec 23:27, Jan 24:40, May 24:24, Oct 24:16
54	**Surgical Care Only:** When 1 physician or other qualified health care professional performs a surgical procedure and another provides preoperative and/or postoperative management, surgical services may be identified by adding modifier 54 to the usual procedure number. *CPT Changes: An Insider's View* 2013 *CPT Assistant* May 24:1, Jan 25:1
55	**Postoperative Management Only:** When 1 physician or other qualified health care professional performed the postoperative management and another performed the surgical procedure, the postoperative component may be identified by adding modifier 55 to the usual procedure number. *CPT Changes: An Insider's View* 2013 *CPT Assistant* Jul 23:14, Jan 25:1
56	**Preoperative Management Only:** When 1 physician or other qualified health care professional performed the preoperative care and evaluation and another performed the surgical procedure, the preoperative component may be identified by adding modifier 56 to the usual procedure number. *CPT Changes: An Insider's View* 2013 *CPT Assistant* Jan 25:1
57	**Decision for Surgery:** An evaluation and management service that resulted in the initial decision to perform the surgery may be identified by adding modifier 57 to the appropriate level of E/M service. *CPT Assistant* May 09:9, Mar 23:1, Jul 23:1
58	**Staged or Related Procedure or Service by the Same Physician or Other Qualified Health Care Professional During the Postoperative Period:** It may be necessary to indicate that the performance of a procedure or service during the postoperative period was: (a) planned or anticipated (staged); (b) more extensive than the original procedure; or (c) for therapy following a surgical procedure. This circumstance may be reported by adding modifier 58 to the staged or related procedure. **Note:** For treatment of a problem that requires a return to the operating/procedure room (eg, unanticipated clinical condition), see modifier 78. *CPT Changes: An Insider's View* 2008, 2013 *CPT Assistant* Nov 22:22, May 24:24, Aug 24:17
59	**Distinct Procedural Service:** Under certain circumstances, it may be necessary to indicate that a procedure or service was distinct or independent from other non-E/M services performed on the same day. Modifier 59 is used to identify procedures/services, other than E/M services, that are not normally reported together, but are appropriate under the circumstances. Documentation must support a different session, different procedure or surgery, different site or organ system, separate incision/excision, separate lesion, or separate injury (or area of injury in extensive injuries) not ordinarily encountered or performed on the same day by the same individual. However, when another already established modifier is appropriate it should be used rather than modifier 59. Only if no more descriptive modifier is available, and the use of modifier 59 best explains the circumstances, should modifier 59 be used. **Note:** Modifier 59 should not be appended to an E/M service. To report a separate and distinct E/M service with a non-E/M service performed on the same date, see modifier 25. *CPT Changes: An Insider's View* 2008 *CPT Assistant* Feb 09:17, Apr 09:4, 8, May 09:6, Jun 09:8, Mar 23:1, Apr 23:24, 26, May 23:24, Jun 23:16, Jul 23:1, Sep 23:1, Oct 23:21, Dec 23:39, Jan 24:17, Jul 24:11,14, Mar 25:18 *Clinical Examples in Radiology* Winter 23:8, Spring 23:20, Summer 23:16, Fall 23:14, Fall 24:3, 12, 13, Winter 25:19
62	**Two Surgeons:** When 2 surgeons work together as primary surgeons performing distinct part(s) of a procedure, each surgeon should report his/her distinct operative work by adding modifier 62 to the procedure code and any associated add-on code(s) for that procedure as long as both surgeons continue to work together as primary surgeons. Each surgeon should report the co-surgery once using the same procedure code. If additional procedure(s) (including add-on procedure[s]) are performed during the same surgical session, separate code(s) may also be reported with modifier 62 added. **Note:** If a co-surgeon acts as an assistant in the performance of additional procedure(s), other than those reported with the modifier 62, during the same surgical session, those services may be reported using separate procedure code(s) with modifier 80 or modifier 82 added, as appropriate. *CPT Changes: An Insider's View* 2013 *CPT Assistant* May 23:27, Jan 24:17
63	**Procedure Performed on Infants less than 4 kg:** Procedures performed on neonates and infants up to a present body weight of 4 kg may involve significantly increased complexity and physician or other qualified health care professional work commonly associated with these patients. This circumstance may be reported by adding modifier 63 to the procedure number. **Note:** Unless otherwise designated, this modifier may only be appended to procedures/services listed in the 20100-69990 code series and 92920, 92928, 92953, 92960, 92986, 92987, 92990, 92997, 92998, 93312, 93313, 93314, 93315, 93316, 93317, 93318, 93452, 93505, 93563, 93564, 93568, 93569, 93573, 93574, 93575, 93580, 93581, 93582, 93590, 93591, 93592, 93593, 93594, 93595, 93596, 93597, 93598, 93615, 93616 from the Medicine/Cardiovascular section. Modifier 63 should not be appended to any CPT codes listed in the **Evaluation and Management Services, Anesthesia, Radiology, Pathology and Laboratory,** or **Medicine** sections (other than those identified above from the Medicine/Cardiovascular section). *CPT Changes: An Insider's View* 2013, 2019, 2020, 2022, 2023 *CPT Assistant* May 23:1, Sep 23:47
66	**Surgical Team:** Under some circumstances, highly complex procedures (requiring the concomitant services of several physicians or other qualified health care professionals, often of different specialties, plus other highly skilled, specially trained personnel,

AAPC: CPT Professional 2026
aapc.vitalsource.com/reader/books/A26BPL0005/epubcfi/6/158[%3Bvnd.vst.idref%3Dt58_Appendix_A]!/4/2[Appendix_A]/10/2/42/4/2/3:110[ans%2C%20or]
Back
Search across book

66	**Surgical Team:** Under some circumstances, highly complex procedures (requiring the concomitant services of several physicians or other qualified health care professionals, often of different specialties, plus other highly skilled, specially trained personnel, various types of complex equipment) are carried out under the "surgical team" concept. Such circumstances may be identified by each participating individual with the addition of modifier 66 to the basic procedure number used for reporting services. *CPT Changes: An Insider's View* 2013
76	**Repeat Procedure or Service by Same Physician or Other Qualified Health Care Professional:** It may be necessary to indicate that a procedure or service was repeated by the same physician or other qualified health care professional subsequent to the original procedure or service. This circumstance may be reported by adding modifier 76 to the repeated procedure or service. **Note:** This modifier should not be appended to an E/M service. *CPT Changes: An Insider's View* 2008, 2011, 2013 *CPT Assistant* Feb 09:6, Jul 23:1
77	**Repeat Procedure by Another Physician or Other Qualified Health Care Professional:** It may be necessary to indicate that a basic procedure or service was repeated by another physician or other qualified health care professional subsequent to the original procedure or service. This circumstance may be reported by adding modifier 77 to the repeated procedure or service. **Note:** This modifier should not be appended to an E/M service. *CPT Changes: An Insider's View* 2008, 2011, 2013 *Clinical Examples in Radiology* Summer 22:10
78	**Unplanned Return to the Operating/Procedure Room by the Same Physician or Other Qualified Health Care Professional Following Initial Procedure for a Related Procedure During the Postoperative Period:** It may be necessary to indicate that another procedure was performed during the postoperative period of the initial procedure (unplanned procedure following initial procedure). When this procedure is related to the first, and requires the use of an operating/procedure room, it may be reported by adding modifier 78 to the related procedure. (For repeat procedures, see modifier 76.) *CPT Changes: An Insider's View* 2008, 2011, 2013 *CPT Assistant* May 24:24, Aug 24:17, Oct 24:13
79	**Unrelated Procedure or Service by the Same Physician or Other Qualified Health Care Professional During the Postoperative Period:** The individual may need to indicate that the performance of a procedure or service during the postoperative period was unrelated to the original procedure. This circumstance may be reported by using modifier 79. (For repeat procedures on the same day, see modifier 76.) *CPT Changes: An Insider's View* 2013
80	**Assistant Surgeon:** Surgical assistant services may be identified by adding modifier 80 to the usual procedure number(s).
81	**Minimum Assistant Surgeon:** Minimum surgical assistant services are identified by adding modifier 81 to the usual procedure number.
82	**Assistant Surgeon (when qualified resident surgeon not available):** The unavailability of a qualified resident surgeon is a prerequisite for use of modifier 82 appended to the usual procedure code number(s).
90	**Reference (Outside) Laboratory:** When laboratory procedures are performed by a party other than the treating or reporting physician or other qualified health care professional, the procedure may be identified by adding modifier 90 to the usual procedure number. *CPT Changes: An Insider's View* 2013
91	**Repeat Clinical Diagnostic Laboratory Test:** In the course of treatment of the patient, it may be necessary to repeat the same laboratory test on the same day to obtain subsequent (multiple) test results. Under these circumstances, the laboratory test performed can be identified by its usual procedure number and the addition of modifier 91. **Note:** This modifier may not be used when tests are rerun to confirm initial results; due to testing problems with specimens or equipment; or for any other reason when a normal, one-time, reportable result is all that is required. This modifier may not be used when other code(s) describe a series of test results (eg, glucose tolerance tests, evocative/suppression testing). This modifier may only be used for laboratory test(s) performed more than once on the same day on the same patient. *CPT Assistant* May 09:6, Sept 13:7
92	**Alternative Laboratory Platform Testing:** When laboratory testing is being performed using a kit or transportable instrument that wholly or in part consists of a single use, disposable analytical chamber, the service may be identified by adding modifier 92 to the usual laboratory procedure code (HIV testing 86701-86703, and 87389). The test does not require permanent dedicated space, hence by its design may be hand carried or transported to the vicinity of the patient for immediate testing at that site, although location of the testing is not in itself determinative of the use of this modifier. *CPT Changes: An Insider's View* 2008, 2012
93	**Synchronous Telemedicine Service Rendered Via Telephone or Other Real-Time Interactive Audio-Only Telecommunications System:** Synchronous telemedicine service is defined as a real-time interaction between a physician or other qualified health care professional and a patient who is located away at a distant site from the physician or other qualified health care professional. The totality of the communication of information exchanged between the physician or other qualified health care professional and the patient during the course of the synchronous telemedicine service must be of an amount and nature that is sufficient to meet the key components and/or requirements of the same service when rendered via a face-to-face interaction. *CPT Changes: An Insider's View* 2023 *CPT Assistant* Sep 24:8, Dec 24:1
95	**Synchronous Telemedicine Service Rendered Via a Real-Time Interactive Audio and Video Telecommunications System:** Synchronous telemedicine service is defined as a **real-time** interaction between a physician or other qualified health care professional and a patient who is located at a distant site from the physician or other qualified health care professional. The totality of the communication of information exchanged between the physician or other qualified health care professional and the patient during the course of the synchronous telemedicine service must be of an amount and nature that would be sufficient to meet the key components and/or requirements of the same service when rendered via a face-to-face interaction. Modifier 95 may only be appended to the services listed in Appendix P. Appendix P is the list of CPT codes for services that are typically performed face-to-face, but may be rendered via a real-time (synchronous) interactive audio and video telecommunications system. *CPT Changes: An Insider's View* 2017 *CPT Assistant* Dec 23:49, Sep 24:8, Dec 24:1, Mar 25:9
96	**Habilitative Services:** When a service or procedure that may be either habilitative or rehabilitative in nature is provided for habilitative purposes, the physician or other qualified health care professional may add modifier 96 to the service or procedure code to indicate that the service or procedure provided was a habilitative service. Habilitative services help an individual learn skills and functioning for daily living that the individual has not yet developed, and then keep and/or improve those learned skills. Habilitative services also help an individual keep, learn, or improve skills and functioning for daily living. *CPT Changes: An Insider's View* 2018
97	**Rehabilitative Services:** When a service or procedure that may be either habilitative or rehabilitative in nature is provided for rehabilitative purposes, the physician or other qualified health care professional may add modifier 97 to the service or procedure code to indicate that the service or procedure provided was a rehabilitative service. Rehabilitative services help an individual keep, get back, or improve skills and functioning for daily living that have been lost or impaired because the individual was sick, hurt, or disabled

AAPC: CPT Professional 2026
aapc.vitalsource.com/reader/books/A26BPL0005/epubcfi/6/158[%3Bvnd.vst.idref%3Dt58_Appendix_A]!/4/2[Appendix_A]/10/2/72/4/2/3:107[suc%2Ch%20s]
Back
Search across book

99	**Multiple Modifiers:** Under certain circumstances 2 or more modifiers may be necessary to completely delineate a service. In such situations modifier 99 should be added to the basic procedure, and other applicable modifiers may be listed as part of the description of the service.

Anesthesia Physical Status Modifiers

The Physical Status modifiers are consistent with the American Society of Anesthesiologists ranking of patient physical status, and distinguishing various levels of complexity of the anesthesia service provided. All anesthesia services are reported by use of the anesthesia five-digit procedure code (00100-01999) with the appropriate physical status modifier appended.

Example: 00100-P1

Under certain circumstances, when another established modifier(s) is appropriate, it should be used in addition to the physical status modifier.

Example: 00100-P4-53

Physical Status Modifier P1: A normal healthy patient

Physical Status Modifier P2: A patient with mild systemic disease

Physical Status Modifier P3: A patient with severe systemic disease

Physical Status Modifier P4: A patient with severe systemic disease that is a constant threat to life

Physical Status Modifier P5: A moribund patient who is not expected to survive without the operation

Physical Status Modifier P6: A declared brain-dead patient whose organs are being removed for donor purposes

Modifiers Approved for Ambulatory Surgery Center (ASC) Hospital Outpatient Use

CPT Level I Modifiers

25	**Significant, Separately Identifiable Evaluation and Management Service by the Same Physician or Other Qualified Health Care Professional on the Same Day of the Procedure or Other Service:** It may be necessary to indicate that on the day a procedure or service identified by a CPT code was performed, the patient's condition required a significant, separately identifiable E/M service above and beyond the other service provided or beyond the usual preoperative and postoperative care associated with the procedure that was performed. A significant, separately identifiable E/M service is defined or substantiated by documentation that satisfies the relevant criteria for the respective E/M service to be reported (see **Evaluation and Management Services Guidelines** for instructions on determining level of E/M service). The E/M service may be prompted by the symptom or condition for which the procedure and/or service was provided. As such, different diagnoses are not required for reporting of the E/M services on the same date. This circumstance may be reported by adding modifier 25 to the appropriate level of E/M service. **Note:** This modifier is not used to report an E/M service that resulted in a decision to perform surgery. See modifier 57. For significant, separately identifiable non-E/M services, see modifier 59. *CPT Changes: An Insider's View* 2008, 2013 *CPT Assistant* Feb 09:22, Mar 09:3, Apr 09:4, Jun 09:11, Mar 23:1, Jul 23:1, Jan 24:1, Sep 24:8, Apr 25:25 *Clinical Examples in Radiology* Spring 23:26
27	**Multiple Outpatient Hospital E/M Encounters on the Same Date:** For hospital outpatient reporting purposes, utilization of hospital resources related to separate and distinct E/M encounters performed in multiple outpatient hospital settings on the same date may be reported by adding modifier 27 to each appropriate level outpatient and/or emergency department E/M code(s). This modifier provides a means of reporting circumstances involving evaluation and management services provided by physician(s) in more than one (multiple) outpatient hospital setting(s) (eg, hospital emergency department, clinic). **Note:** This modifier is not to be used for physician reporting of multiple E/M services performed by the same physician on the same date. For physician reporting of all outpatient evaluation and management services provided by the same physician on the same date and performed in multiple outpatient setting(s) (eg, hospital emergency department, clinic), see **Evaluation and Management, Emergency Department,** or **Preventive Medicine Services** codes.
33	►**Preventive Services:** When the primary purpose of the service is the delivery of an evidence-based service in accordance with a US Preventive Services Task Force A or B rating in effect or is another preventive service identified in preventive services mandates (legislative or regulatory), the service may be identified by adding modifier 33 to the procedure. For separately reported services specifically identified as preventive, the modifier should not be used.◄ *CPT Changes: An Insider's View* 2012, 2026 *CPT Assistant* Sep 24:8
50	**Bilateral Procedure:** Unless otherwise identified in the listings, bilateral procedures that are performed at the same session should be identified by adding modifier 50 to the appropriate 5 digit code. **Note:** This modifier should not be appended to designated "add-on" codes (see Appendix D). *CPT Changes: An Insider's View* 2011, 2020 *CPT Assistant* Apr 09:9, Jun 13:15, Dec 22:20, Mar 23:32, May 23:28, Jul 23:18, Aug 23:14, Sep 23:1, Oct 23:19, Jan 24:17, Feb 24:34, May 24:7, Aug 24:1, Dec 24:58, Feb 25:29 *Clinical Examples in Radiology* Winter 23:8, Fall 23:32, Winter 24:17, Spring 24:3,25, Summer 24:3, Fall 24:4, 12, Winter 25:19
52	**Reduced Services:** Under certain circumstances a service or procedure is partially reduced or eliminated at the discretion of the physician or other qualified health care professional. Under these circumstances the service provided can be identified by its usual procedure number and the addition of modifier 52, signifying that the service is reduced. This provides a means of reporting reduced services without disturbing the identification of the basic service. **Note:** For hospital outpatient reporting of a previously scheduled procedure/service that is partially reduced or cancelled as a result of extenuating circumstances or those that threaten the well-being of the patient prior to or after administration of anesthesia, see modifiers 73 and 74 (see modifiers approved for ASC hospital outpatient use). *CPT Changes: An Insider's View* 2013 *CPT Assistant* Mar 09:11, Apr 09:5, May 09:8, Jun 09:10, Jun 13:17, Nov 22:22, Dec 22:20, Feb 23:6, May 23:28, Jun 23:25, Dec 23:23, Jan 24:17, Apr 24:24, Jul 24:15, Aug 24:14, Sep 24:26, Oct 24:14, Dec 24:53, Jan 25:32 *Clinical Examples in Radiology* Summer 23:23, Winter 24:30, Summer 24:7, Winter 25:27

AAPC: CPT Professional 2026
aapc.vitalsource.com/reader/books/A26BPL0005/epubcfi/6/158[%3Bvnd.vst.idref%3Dt58_Appendix_A]!/4/2[Appendix_A]/38/2/12/4/2/2/1:118[%20Po%2Csto]
Back
Search across book

58	**Staged or Related Procedure or Service by the Same Physician or Other Qualified Health Care Professional During the Postoperative Period:** It may be necessary to indicate that the performance of a procedure or service during the postoperative period was: (a) planned or anticipated (staged); (b) more extensive than the original procedure; or (c) for therapy following a surgical procedure. This circumstance may be reported by adding modifier 58 to the staged or related procedure. **Note:** For treatment of a problem that requires a return to the operating/procedure room (eg, unanticipated clinical condition), see modifier 78. *CPT Changes: An Insider's View* 2008, 2013 *CPT Assistant* Nov 22:22, May 24:24, Aug 24:17
59	**Distinct Procedural Service:** Under certain circumstances, it may be necessary to indicate that a procedure or service was distinct or independent from other non-E/M services performed on the same day. Modifier 59 is used to identify procedures/services, other than E/M services, that are not normally reported together, but are appropriate under the circumstances. Documentation must support a different session, different procedure or surgery, different site or organ system, separate incision/excision, separate lesion, or separate injury (or area of injury in extensive injuries) not ordinarily encountered or performed on the same day by the same individual. However, when another already established modifier is appropriate it should be used rather than modifier 59. Only if no more descriptive modifier is available, and the use of modifier 59 best explains the circumstances, should modifier 59 be used. **Note:** Modifier 59 should not be appended to an E/M service. To report a separate and distinct E/M service with a non-E/M service performed on the same date, see modifier 25. *CPT Changes: An Insider's View* 2008 *CPT Assistant* Feb 09:17, Apr 09:4, 8, May 09:6, Jun 09:8, Mar 23:1, Apr 23:24, 26, May 23:24, Jun 23:16, Jul 23:1, Sep 23:1, Oct 23:21, Dec 23:39, Jan 24:17, Jul 24:11,14, Mar 25:18 *Clinical Examples in Radiology* Winter 23:8, Spring 23:20, Summer 23:16, Fall 23:14, Fall 24:3, 12, 13, Winter 25:19
73	**Discontinued Out-Patient Hospital/Ambulatory Surgery Center (ASC) Procedure Prior to the Administration of Anesthesia:** Due to extenuating circumstances or those that threaten the well being of the patient, the physician may cancel a surgical or diagnostic procedure subsequent to the patient's surgical preparation (including sedation when provided, and being taken to the room where the procedure is to be performed), but prior to the administration of anesthesia (local, regional block(s) or general). Under these circumstances, the intended service that is prepared for but cancelled can be reported by its usual procedure number and the addition of modifier 73. **Note:** The elective cancellation of a service prior to the administration of anesthesia and/or surgical preparation of the patient should not be reported. For physician reporting of a discontinued procedure, see modifier 53. *CPT Assistant* Jan 23:31, May 24:24, Oct 24:16
74	**Discontinued Out-Patient Hospital/Ambulatory Surgery Center (ASC) Procedure After Administration of Anesthesia:** Due to extenuating circumstances or those that threaten the well being of the patient, the physician may terminate a surgical or diagnostic procedure after the administration of anesthesia (local, regional block(s), general) or after the procedure was started (incision made, intubation started, scope inserted, etc). Under these circumstances, the procedure started but terminated can be reported by its usual procedure number and the addition of modifier 74. **Note:** The elective cancellation of a service prior to the administration of anesthesia and/or surgical preparation of the patient should not be reported. For physician reporting of a discontinued procedure, see modifier 53. *CPT Assistant* Jun 13:17, Jan 23:31, May 24:24, Oct 24:16
76	**Repeat Procedure or Service by Same Physician or Other Qualified Health Care Professional:** It may be necessary to indicate that a procedure or service was repeated by the same physician or other qualified health care professional subsequent to the original procedure or service. This circumstance may be reported by adding modifier 76 to the repeated procedure or service. **Note:** This modifier should not be appended to an E/M service. *CPT Changes: An Insider's View* 2008, 2011, 2013 *CPT Assistant* Feb 09:6, Jul 23:1
77	**Repeat Procedure by Another Physician or Other Qualified Health Care Professional:** It may be necessary to indicate that a basic procedure or service was repeated by another physician or other qualified health care professional subsequent to the original procedure or service. This circumstance may be reported by adding modifier 77 to the repeated procedure or service. **Note:** This modifier should not be appended to an E/M service. *CPT Changes: An Insider's View* 2008, 2011, 2013 *Clinical Examples in Radiology* Summer 22:10
78	**Unplanned Return to the Operating/Procedure Room by the Same Physician or Other Qualified Health Care Professional Following Initial Procedure for a Related Procedure During the Postoperative Period:** It may be necessary to indicate that another procedure was performed during the postoperative period of the initial procedure (unplanned procedure following initial procedure). When this procedure is related to the first, and requires the use of an operating/procedure room, it may be reported by adding modifier 78 to the related procedure. (For repeat procedures, see modifier 76.) *CPT Changes: An Insider's View* 2008, 2011, 2013 *CPT Assistant* May 24:24, Aug 24:17, Oct 24:13
79	**Unrelated Procedure or Service by the Same Physician or Other Qualified Health Care Professional During the Postoperative Period:** The individual may need to indicate that the performance of a procedure or service during the postoperative period was unrelated to the original procedure. This circumstance may be reported by using modifier 79. (For repeat procedures on the same day, see modifier 76.) *CPT Changes: An Insider's View* 2013
91	**Repeat Clinical Diagnostic Laboratory Test:** In the course of treatment of the patient, it may be necessary to repeat the same laboratory test on the same day to obtain subsequent (multiple) test results. Under these circumstances, the laboratory test performed can be identified by its usual procedure number and the addition of modifier 91. **Note:** This modifier may not be used when tests are rerun to confirm initial results; due to testing problems with specimens or equipment; or for any other reason when a normal, one-time, reportable result is all that is required. This modifier may not be used when other code(s) describe a series of test results (eg, glucose tolerance tests, evocative/suppression testing). This modifier may only be used for laboratory test(s) performed more than once on the same day on the same patient. *CPT Assistant* May 09:6, Sept 13:7

Category II Modifiers

The following performance measurement modifiers may be used for Category II codes to indicate that a service specified in the associated measure(s) was considered but, due to either medical, patient, or system circumstance(s) documented in the medical record, the service was not provided. These modifiers serve as denominator exclusions from the performance measure. The user should note that not all listed measures provide for exclusions (see Alphabetical Clinical Topics Listing for more discussion regarding exclusion criteria).

Category II modifiers should only be reported with Category II codes—they should not be reported with Category I or Category III codes. In addition, the modifiers in the Category II section should only be used where specified in the guidelines, reporting instructions, parenthetic notes, or code descriptor language listed in the Category II section (code listing and the Alphabetical Clinical Topics Listing).

1P Performance Measure Exclusion Modifier due to Medical Reasons:

974 / 1346

AAPC: CPT Professional 2026
aapc.vitalsource.com/reader/books/A26BPL0005/epubcfi/6/158[%3Bvnd.vst.idref%3Dt58_Appendix_A]!/4/2[Appendix_A]/40/2
Work
Back
Search across book

Category II Modifiers

The following performance measurement modifiers may be used for Category II codes to indicate that a service specified in the associated measure(s) was considered but, due to either medical, patient, or system circumstance(s) documented in the medical record, the service was not provided. These modifiers serve as denominator exclusions from the performance measure. The user should note that not all listed measures provide for exclusions (see Alphabetical Clinical Topics Listing for more discussion regarding exclusion criteria).

Category II modifiers should only be reported with Category II codes—they should not be reported with Category I or Category III codes. In addition, the modifiers in the Category II section should only be used where specified in the guidelines, reporting instructions, parenthetic notes, or code descriptor language listed in the Category II section (code listing and the Alphabetical Clinical Topics Listing).

1P Performance Measure Exclusion Modifier due to Medical Reasons:

Reasons include:

- Not indicated (absence of organ/limb, already received/performed, other)
- Contraindicated (patient allergic history, potential adverse drug interaction, other)
- Other medical reasons

2P Performance Measure Exclusion Modifier due to Patient Reasons:

Reasons include:

- Patient declined
- Economic, social, or religious reasons
- Other patient reasons

3P Performance Measure Exclusion Modifier due to System Reasons:

Reasons include:

- Resources to perform the services not available
- Insurance coverage/payor-related limitations
- Other reasons attributable to health care delivery system

Modifier 8P is intended to be used as a "reporting modifier" to allow the reporting of circumstances when an action described in a measure's numerator is not performed and the reason is not otherwise specified.

8P Performance measure reporting modifier–action not performed, reason not otherwise specified

Level II (HCPCS/National) Modifiers

E1	Upper left, eyelid
E2	Lower left, eyelid
E3	Upper right, eyelid
E4	Lower right, eyelid
FA	Left hand, thumb
F1	Left hand, second digit
F2	Left hand, third digit
F3	Left hand, fourth digit
F4	Left hand, fifth digit
F5	Right hand, thumb
F6	Right hand, second digit
F7	Right hand, third digit
F8	Right hand, fourth digit
F9	Right hand, fifth digit
GG	Performance and payment of a screening mammogram and diagnostic mammogram on the same patient, same day

AAPC: CPT Professional 2026
aapc.vitalsource.com/reader/books/A26BPL0005/epubcfi/6/158[%3Bvnd.vst.idref%3Dt58_Appendix_A]!/4/2[Appendix_A]/70/2/42/4/2/2
Back
Search across book

E2	Lower left, eyelid
E3	Upper right, eyelid
E4	Lower right, eyelid
FA	Left hand, thumb
F1	Left hand, second digit
F2	Left hand, third digit
F3	Left hand, fourth digit
F4	Left hand, fifth digit
F5	Right hand, thumb
F6	Right hand, second digit
F7	Right hand, third digit
F8	Right hand, fourth digit
F9	Right hand, fifth digit
GG	Performance and payment of a screening mammogram and diagnostic mammogram on the same patient, same day
GH	Diagnostic mammogram converted from screening mammogram on same day
LC	Left circumflex coronary artery
LD	Left anterior descending coronary artery
LM	Left main coronary artery
LT	Left side (used to identify procedures performed on the left side of the body)
QM	Ambulance service provided under arrangement by a provider of services
QN	Ambulance service furnished directly by a provider of services
RC	Right coronary artery
RI	Ramus intermedius coronary artery
RT	Right side (used to identify procedures performed on the right side of the body)
TA	Left foot, great toe
T1	Left foot, second digit
T2	Left foot, third digit
T3	Left foot, fourth digit
T4	Left foot, fifth digit
T5	Right foot, great toe
T6	Right foot, second digit
T7	Right foot, third digit
T8	Right foot, fourth digit
T9	Right foot, fifth digit
XE	Separate Encounter*
XP	Separate Practitioner*
XS	Separate Organ/Structure*
XU	Unusual Separate Service*

(*HCPCS modifiers for selective identification of subsets of Distinct Procedural Services [59 modifier])

975 / 1346